APEx DELIVERABLE 2

Digital Mental Health Intervention

Onboarding & Engagement Toolkit

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PRODUCT	A user-facing communication and onboarding package for a proposed digital mental-health intervention for underserved U.S. adolescents.

1. Product Overview

This deliverable converts the project's needs assessment and RDoC-informed framework into practical, adolescent-facing materials. It is a design prototype and not a validated clinical tool.

Purpose

- Explain the tool in plain language to adolescents.
- Offer onboarding that does not start with a diagnostic label.
- Show how population findings can shape intervention content.
- Prototype one RDoC-informed content pathway for later evaluation.
- Keep access and referral assumptions explicit (no assumed insurance or clinical access).

2. Design Rationale

The needs assessment finds elevated depression and unmet treatment need among low-income U.S. adolescents, plus greater insurance instability. These findings drive the product decisions below.

Evidence / finding	Design implication	Product response
Higher depression and conduct problems in the lower-income group	Prioritize low-burden mood, motivation, and coping support	Starter module: small activity when reward/motivation feels low
Greater unmet mental-health treatment need	Do not assume timely clinical access	Tool framed as support/information; separate pathway for additional help
Insurance instability more common in the target group	Avoid insurance as a prerequisite for engagement	Core onboarding collects no insurance information
Anxiety common but not disproportionately concentrated by income	Include anxiety support without centering equity rationale on it	Anxiety appears as an optional check-in pathway
Diagnostic labels do not map cleanly to functional processes	Use dimensional, experience-based check-ins	Users describe experience before any interpretation is offered
RDoC constructs and digital signals require validation	Do not present inferred constructs as diagnoses or biomarkers	Cautious language: "may be an important part of what you're experiencing"

3. User-Facing Explainer *Intended for a one-page digital flyer, landing page, or recruitment/onboarding screen.*

FEELING STUCK? YOU DON'T HAVE TO FIGURE IT OUT ALONE.

A digital mental-health support tool to help you check in with yourself, notice patterns in how you're feeling, and try small strategies that may help.

What you can do here

- Check in about how you're feeling and what's been hard lately.
- Explore patterns in mood, motivation, stress, sleep, and connection.

- Try short, practical activities and notice how they affect you.
- Return later and see what changes over time.

What this tool is not

- It is not a diagnosis.
- It does not replace a therapist, doctor, or other professional care.
- Your responses do not define who you are.
- It makes no clinical claims from unvalidated digital signals.

Need more support? The tool offers information and self-guided activities. Some situations require a trusted person or qualified professional. A separate support screen should provide locally appropriate resources and urgent-safety guidance.

4. Mobile Onboarding Prototype

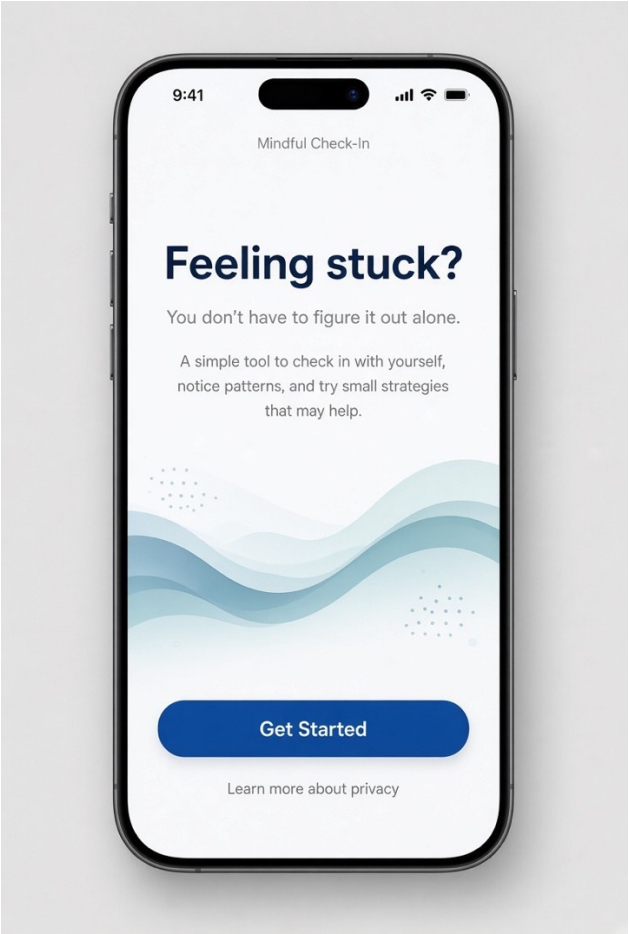
Five-screen minimum viable sequence. Intentionally simple, low-literacy, and mobile-first. Test for clarity, acceptability, accessibility, and cultural fit before use.

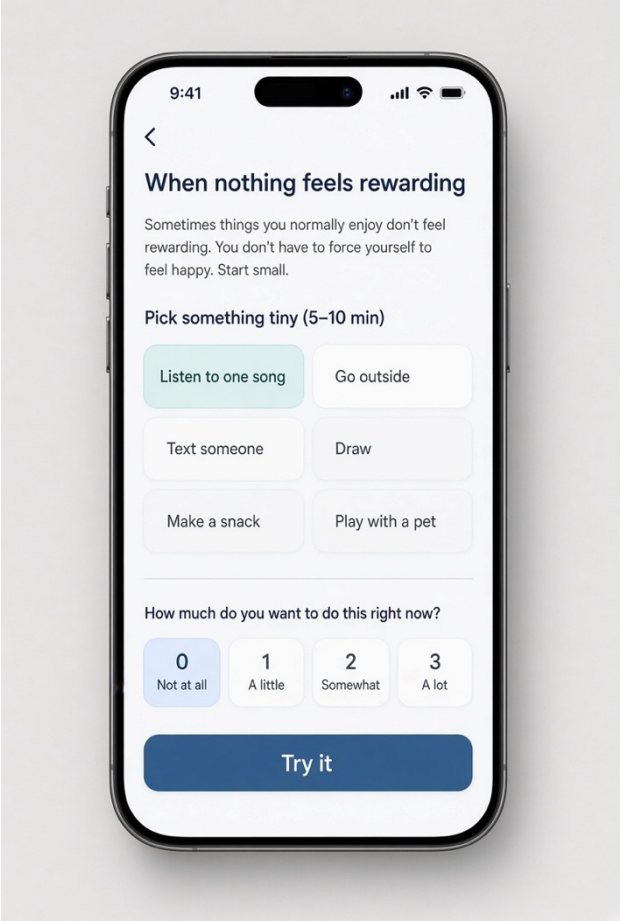
Screen	Purpose & content
01 Welcome	Establishes the tool as supportive, not diagnostic → Get Started.
02 Privacy	Explains what information is collected, why, who can access it, and how long it is retained
03 Check-in	User describes experience using plain-language categories rather than a diagnostic label.
04 Explore	Shifts from “What disorder do you have?” toward “What experience is most relevant right now?” (RDoC-informed framing).
05 Support	Introduces a low-burden starter activity; reinforces that the user does not need to complete it perfectly.

5. Scoped Content Module

Brief, low-resource activity pathway linked to the needs assessment’s focus on depression and to the RDoC Positive Valence Systems construct (reward responsiveness)

Screen	Content
A	Name the experience. Sometimes things you normally enjoy don’t feel rewarding. You don’t have to force yourself to feel happy. The first step can be noticing what is happening.
B	Pick something tiny (5–10 min): listen to one song, go outside, text someone, draw, make a snack, play with a pet. Stop whenever you want.
C	Before you try it: How much do you want to do this right now? 0 — Not at all | 1 — A little | 2 — Somewhat | 3 — A lot
D	Try it. Give it a few minutes. No requirement to finish. Goal: notice your experience, not perform.
E	Check in again: Worse | About the same | A little better | Much better. Optional: “What did you notice?”
F	Reflect. One small change is still information. Try something different next time. If things stay difficult, reach out to someone you trust or seek professional support.





6. Safety, Privacy & Equity Guardrails

The prototype follows strict guardrails: no diagnostic language and no claims that digital signals (typing, speech, app use) are validated biomarkers; only the minimum data needed for the proposed function is collected, with transparent explanation of what is collected, why, who can access it, and how long it is retained; a clear path to trusted adults, professional support, and urgent-safety resources is always available; culturally variable communication or social behavior is never treated as inherently abnormal; core content does not assume insurance or a guaranteed clinical referral; and all adolescent-facing material requires usability, acceptability, feasibility, safety, and cultural review before any real-world use.

Sources

1. NSCH Needs assessment prepared for the NYU Langone Health Department of Population Health
2. RDoC as a Roadmap for Precision AI in Mental Health: From Diagnosis to Neurobiologically Informed Treatment (project paper).